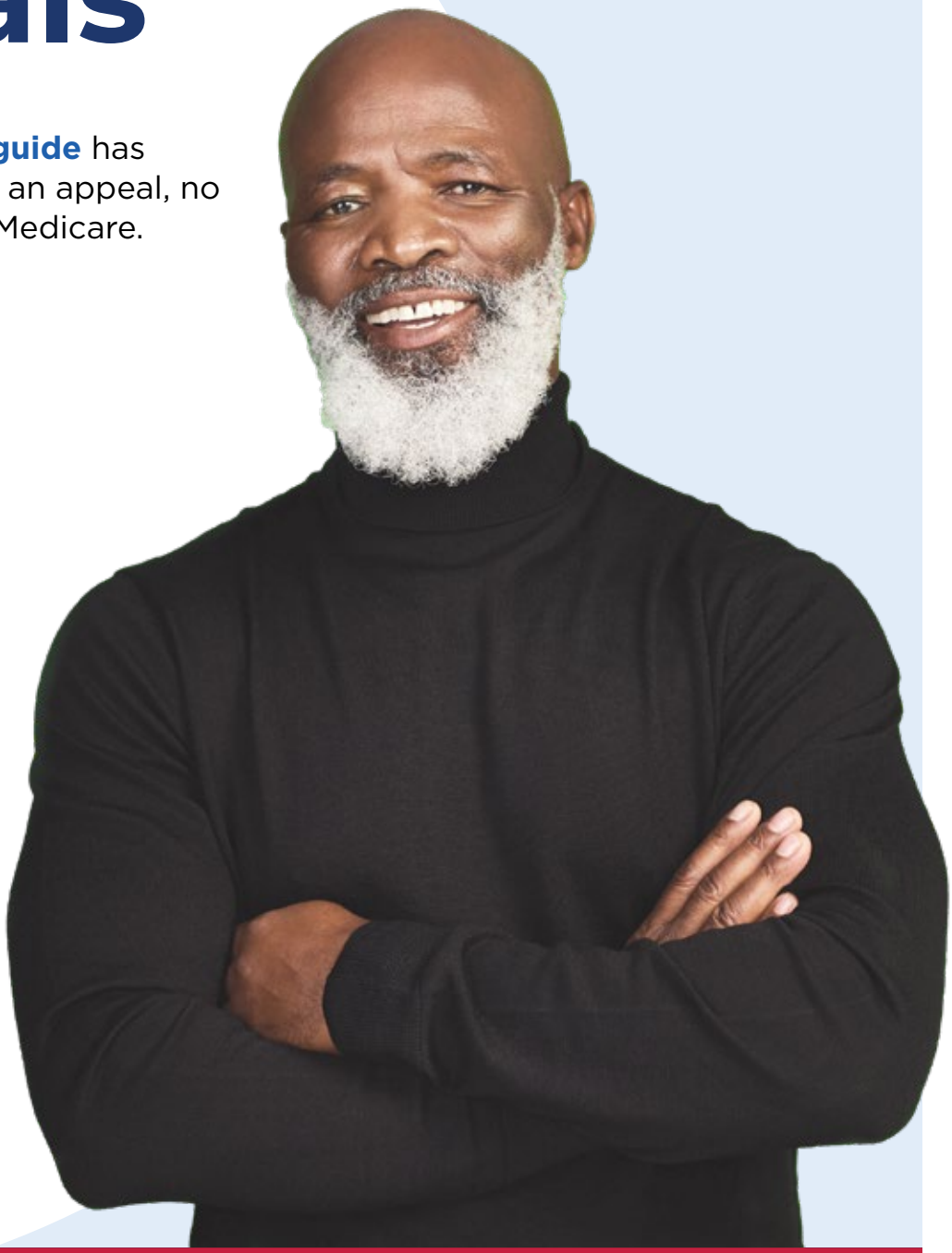
Medicare Appeals

This **official government guide** has information on how to file an appeal, no matter how you get your Medicare.

Medicare.gov



Medicare

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Section 1:

File an appeal

If you disagree with a coverage or payment decision by **Original Medicare**, your **Medicare Advantage** or other Medicare health plan, or your **Medicare drug plan** you can file an **appeal**. Before you start an appeal, ask your provider or supplier for any information that will make your appeal stronger.

If you're in a Medicare Advantage Plan, other health plan, or a drug plan, check your plan materials, or contact your plan, for details about your appeal rights. Your plan must tell you, in writing, how to appeal. Generally, you can find your plan's contact information on your plan membership card.

Note: Go to pages 40–43 for definitions of **blue** words

You can file an appeal if Medicare or your plan refuses to:

- Cover a health care service, supply, item, or drug you think should be covered.
- Pay for a health care service, supply, item, or drug you already got.
- Change the amount you must pay for a health care service, supply, item, or drug.
- Provide or pay for all or part of a health care service, supply, item, or drug you think you still need.
- Cover drugs like opioids and benzodiazepines. This can happen when the plan has labeled you "at-risk" under a Part D drug management program.

How do appeals work?

The appeals process varies based on the kind of coverage you have. Generally, there are 5 levels of appeals. If you disagree with the decision made at any level of the process, you can usually go to the next level. At each level you'll get a decision letter with instructions on how to move to the next level of appeal.

- To learn how to file an appeal in **Original Medicare**, go to page 5.
- To learn how to file an appeal in a **Medicare Advantage Plan** or other Medicare health plan, go to page 17.
- To learn how to file an appeal with a Medicare drug coverage plan, go to page 30.



Section 2:

Appoint a representative

If you want help filing an appeal, you can appoint a representative to help you. Your representative can be a family member, friend, advocate, attorney, doctor, or someone else that acts on your behalf.

There are 2 ways that you can appoint a representative to help you with an appeal. You can:

- Fill out an "Appointment of Representative" form. You can download different formats of this form at CMS.gov/Medicare/CMS-Forms/CMS-Forms/downloads/cms1696.pdf.
  - After you fill out this form, send it with your appeal to the Medicare Administrative Contractor (MAC) (a company that handles Original Medicare claims) that's listed on your Medicare Summary Notice (MSN), or your plan.

Note: Go to pages 40–43 for definitions of blue words

OR

- Submit a written request that includes:
  - Your name, address, phone number, and Medicare Number.
  - A statement appointing someone as your representative to act on your behalf.
  - The name, address, and phone number of your representative.
  - The professional status of your representative (like a doctor) or their relationship to you.
  - A statement authorizing the release of your personal and identifiable health information to your representative.
  - A statement explaining why you're being represented and to what extent.

Send the written request with your appeal to the Medicare Administrative Contractor (MAC) (a company that handles **Original Medicare** claims) that's listed on your **MSN** or to your plan.

Keep a copy of everything you send as part of your appeal.

If you have questions about appointing a representative, call 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.

In some cases, your doctor can file an appeal on your behalf, without being appointed as your representative.

- If you have a **Medicare Advantage Plan** or other Medicare health plan:
  - Your treating doctor can ask for an organization determination or certain pre-service reconsiderations on your behalf, and you don't need to submit an "Appointment of Representative" form or written request.
  - If you want your treating doctor to ask for a higher level of appeal on your behalf, you'll need to submit the "Appointment of Representative" form or a written request that includes the information listed above.
- If you have a **Medicare drug plan**:
  - Your doctor or other prescriber can ask for a coverage determination, redetermination, or reconsideration from the Independent Review Entity (IRE) on your behalf, and you don't need to submit an "Appointment of Representative" form.
  - If you want your doctor or other prescriber to ask for a higher level of appeal on your behalf, you'll need to submit the "Appointment of Representative" form or a written request that includes the information listed above.



Section 3:

File an appeal in Original Medicare

Original Medicare includes **Medicare Part A (Hospital Insurance)** and **Medicare Part B (Medical Insurance)**. Everyone with Original Medicare who gets covered services will get a **"Medicare Summary Notice" (MSN)** in the mail at least every 6 months. If you want to get electronic MSNs, log in (or create) your secure Medicare account at **Medicare.gov/account/login** and go to account settings to sign up for electronic MSNs. If you sign up for eletronic MSNs, you'll get an email with a link to your MSN for any month you have a processed claim instead of waiting for a paper copy.

Note: Go to pages 40–43 for definitions of **blue** words

Your **MSN** shows:

- All items and services that providers and suppliers billed to Medicare during the 6-month period
- What Medicare paid
- What you may owe the provider or supplier
- If Medicare has approved, or fully or partially denied, your medical **claim** (this is the initial determination, and it's made by the Medicare Administrative Contractor (MAC), which processes Medicare claims)

Read your MSN carefully. If you disagree with a Medicare coverage or payment decision, you can **appeal** the decision. Your MSN has information on how to file an appeal.

If you decide to file an appeal, ask your doctor, other **health care provider**, or supplier for any information that may help your case. Keep a copy of everything you send to Medicare as part of your appeal.

Contact your provider or supplier if an expected item or service is missing on your MSN, or you aren't sure if Medicare was billed for the items and services you got. Ask them for an itemized statement that lists all of the items and services they billed to Medicare.

Note: Your provider has up to one year from the date you got an item or service to submit a claim.

What's the appeals process for Original Medicare?

There are 5 levels of appeal in **Original Medicare**. If you disagree with the decision made at any level of the process, you can usually go to the next level. At each level you'll get a decision letter with instructions on how to move to the next level. The 5 levels are:

Level 1: Redetermination by the MAC

Level 2: Reconsideration by a Qualified Independent Contractor (QIC)

Level 3: Decision by the Office of Medicare Hearings and Appeals (OMHA)

Level 4: Review by the Medicare Appeals Council (Appeals Council)

Level 5: Judicial Review by a Federal District Court

Level 1: Redetermination by the MAC

The first level of appeal in Original Medicare is called a redetermination.

Start your appeal by looking at your MSN. You must file your appeal by the date listed in your MSN. If you miss the deadline for appealing, you may still file an appeal and get a decision **if** you can show good cause for missing the deadline (for example, if you have a disability, illness, or accident that delayed you from filing your appeal by the deadline).

How do I file a redetermination?

Read your MSN carefully, and follow the appeal instructions:

- Circle the item(s) and/or service(s) you wish to appeal on a copy of your MSN.
- Explain in writing on your MSN or a separate piece of paper why you disagree with the initial determination.
- Include your name, phone number, and Medicare Number.
- Include any other information you have about your appeal. You can ask your provider for any information that may help your case. Write your Medicare Number on all documents you submit with your appeal request.
- Mail your documents to the Medicare Claims office address listed in Step 7 on the last page of your MSN.

Or you can fill out a "Medicare Redetermination Request" form (CMS Form number 20027).

- To get a copy, visit CMS.gov/cmsforms/downloads/cms20027.pdf, or call 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.
- Include any other information you have about your appeal. You can ask your provider for any information that may help your case. Write your Medicare Number on all documents you submit with your appeal request.
- Mail your documentation to the Medicare Claims office address listed in Step 7 on the last page of your MSN.

Or send a written request to the MAC. The company's address is listed on your MSN. Your request must include:

- Your name and Medicare Number.
- List the specific items and/or services and dates you're filing an appeal about. You can also circle the items and/or services you wish to appeal on a copy of your MSN.
- Why you think the items and/or services should be covered.
- If you've appointed a representative, a copy of the "Appointment of Representative" form or any written instrument that has the required elements. For more information on appointing a representative, go to page 3.
- Any other information that may help your case.
- Mail your documents to the Medicare Claims office address listed in Step 7 on the last page of your MSN.

Keep a copy of everything you send to Medicare as part of your appeal. You'll generally get a decision from the MAC (either in a letter or an MSN) within 60 days after they get your appeal.

Medicare contracts with the MAC to review your appeal and make a decision. The people at the MAC who review and decide on your appeal are separate from the people who made the original coverage or payment decision.

You can send more information or evidence to the MAC after filing your redetermination request, but it may take longer than 60 days for the MAC to make a decision. The MAC will get an extra 14 calendar days to make a decision each time you send them more information or evidence.

After you file a redetermination

- If the MAC decides that Medicare **will** cover the item(s) or service(s) you appealed: Your next MSN will show the decision.
- If the MAC decides that Medicare **won't** cover the appealed item(s) or service(s): You'll generally get a decision letter from the MAC within 60 days.

If you disagree with the MAC's decision:

You have 180 days after you get the MAC's decision letter or an MSN to ask for a Level 2 appeal, called a "Reconsideration" by a Qualified Independent Contractor (QIC).

Level 2: Qualified Independent Contractor (QIC) Reconsideration

A QIC is an independent contractor that didn't take part in the Level 1 decision. The QIC will review your request for a reconsideration and make a decision. Your request should clearly explain why you disagree with the redetermination decision from Level 1. It's helpful to send a copy of the "Medicare Redetermination Notice" with your request for a reconsideration to the QIC.

How do I ask for a QIC reconsideration?

To ask for a reconsideration, follow the directions on the "Medicare Redetermination Notice" you got in Level 1. You must send your request to the QIC's address listed on the "Medicare Redetermination Notice." You can ask for a reconsideration in **one of these ways**:

- **Fill out a "Medicare Reconsideration Request" form**, which is included with the "Medicare Redetermination Notice."
  - You can also get a copy of this form by visiting **CMS.gov/cmsforms/downloads/cms20033.pdf**, or calling 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.

- **Send a written request that includes:**
  - Your name and Medicare Number.
  - The specific item(s) and/or service(s) you're requesting a reconsideration for and the specific date(s) of service. You can find this information in your "Medicare Redetermination Notice."
  - The name of the MAC that made the redetermination, which you can find on your MSN and on the "Medicare Redetermination Notice."
  - An explanation of why you disagree with the redetermination decision, and any evidence that might help make your appeal stronger.
  - The name of your representative, if you've appointed one. For more information on appointing a representative, go to section 2.
  - Mail your request with any additional documents to the QIC's address listed on the "Medicare Redetermination Notice".

No matter how you choose to ask for a reconsideration, you should clearly explain why you disagree with the redetermination decision from Level 1. Send a copy of the "Medicare Redetermination Notice" with your request for a reconsideration to the QIC. Information you submitted at Level 1 will be sent to the QIC to review. You can also send the QIC any more information that may help your case. If you send them information or evidence after your reconsideration request has been filed, it may take longer for the QIC to make a decision. **Keep a copy of everything you send to Medicare as part of your** appeal.

In most cases, the QIC will send you a written reconsideration decision about 60 days after the QIC gets your appeal request (with exceptions for extensions if you send in more evidence or file late). If the QIC is unable to complete its reconsideration in 60 days, you may ask for your case to move to the next level of appeal. If this happens, the QIC will send you a notice that explains your options: (1) to wait for the QIC to issue a decision, or (2) to send your appeal to the Office of Medicare Hearings and Appeals (OMHA). If you choose to send your appeal to OMHA, a written request must be filed with the QIC. The QIC can help you with what's needed for this request.

If you disagree with the reconsideration decision in Level 2, you have 60 days after you get the QIC reconsideration decision letter to ask for a decision by OMHA, which is Level 3.

Level 3: Decision by the Office of Medicare Hearings and Appeals (OMHA)

If you aren't satisfied with the QIC reconsideration decision, you can ask OMHA for a hearing before an Administrative Law Judge (ALJ) or, in certain circumstances, a review of your appeal record by an ALJ or attorney adjudicator.

A hearing before an ALJ allows you to present your appeal to a new person who will independently review the facts of your appeal and listen to your testimony before making a decision. An ALJ hearing is usually held by phone or video-teleconference, but can be held in person if the ALJ finds that you have a good reason.

You can also ask OMHA to make a decision without holding a hearing (based only on the information that's in your appeal record). If you do this, either an ALJ or an attorney adjudicator will review the information in your appeal record and issue a decision. The ALJ or attorney adjudicator may also issue a decision without holding a hearing if, for example, information in your appeal record supports a decision that's fully in your favor.

To get a hearing or review by OMHA, the amount of your case must meet a minimum dollar amount. The minimum dollar amount for 2026 is $200. The QIC reconsideration decision letter may include a statement that tells you if your case is estimated to meet the minimum dollar amount. However, it's up to the ALJ to make the final decision. **You may be able to combine claims to meet the minimum dollar amount.**

How do I ask for an appeal by an Administrative Law Judge (ALJ)?

Follow the directions on the QIC reconsideration decision letter you got from the QIC in Level 2 to ask OMHA for a hearing before an ALJ. You must send your request to the OMHA Central Operations. The address is listed in the QIC's reconsideration notice. You or your representative can ask for a hearing in **one of these ways**:

- Fill out a "Request for Administrative Law Judge Hearing or Review of Dismissal" form (OMHA-100), which is included with the QIC reconsideration decision letter. You can also get a copy by visiting HHS.gov/about/agencies/omha/filing-an-appeal/forms, or calling 1-800-MEDICARE (1-800-633-4227).
- Send a written request that includes:
  - Your name, address, phone number, and Medicare Number. If you've appointed a representative, include their name, address, and phone number.
  - The appeal number included on the QIC reconsideration decision, if any.
  - The dates of service for the items or services you're appealing. Your MSN or QIC reconsideration decision letter will have this information.
  - Why you disagree with the reconsideration decision you're appealing.
  - Any information that may help your case. If you can't include this information with your request, include a statement explaining what you plan to submit and when you'll submit it.

How do I ask for a review of my case without a hearing?

You can ask OMHA to make a decision without holding a hearing based only on the information in your appeal record. To do this, include one of these in your appeal request to OMHA:

- The "Waiver of Right to an Administrative Law Judge (ALJ) Hearing" form (Form OMHA-104). You can get a copy by visiting HHS.gov/about/agencies/omha/filing-an-appeal/forms.
- A written request stating you don't wish to appear before an ALJ at a hearing (including a hearing held by phone or video-teleconference), and why you want to waive the hearing.

Even if you want to waive the ALJ hearing, a hearing may still be held by an ALJ if the other parties in your case (for example, your provider) don't also waive the ALJ hearing, or if the ALJ believes a hearing is necessary to decide your case.

If you have asked OMHA for a decision without a hearing, but the ALJ decides a hearing is necessary, the ALJ will let you know when the hearing will be. If no hearing is held, either an ALJ or attorney adjudicator will review the information in your appeal record and issue a decision.

Keep a copy of everything you send to Medicare as part of your appeal. For more information about the OMHA decision process, visit HHS.gov/about/agencies/omha/filing-an-appeal/coverage-and-claims-appeals. If you need help filing an appeal with an ALJ, call 1-800-MEDICARE.

If the ALJ doesn't issue a timely decision, you may ask OMHA to move the case to the next level of appeal by sending a request to OMHA at the address listed on the QIC reconsideration notice.

If you disagree with OMHA's decision, you may ask for a review at the next level of appeal by the Medicare Appeals Council. You have 60 days after you get OMHA's decision to ask for an appeal at level 4.

Level 4: Review by the Medicare Appeals Council

You can ask for the Medicare Appeals Council to review your case, regardless of the dollar amount.

How do I ask for a review by the Medicare Appeals Council?

To ask for a Level 4 appeal, follow the directions in the OMHA's hearing decision you got in the Level 3 appeal.

You can ask for a review by the Medicare Appeals Council in 1 of 2 ways:

- Fill out a **"Request for Review of an Administrative Law Judge (ALJ) Medicare Decision/Dismissal"** form (DAB-101). To get a copy, visit HHS.gov/about/agencies/dab/different-appeals-at-dab/appeals-to-council/form, or call 1-800-MEDICARE to ask for a copy.

- Send a written request to the Medicare Appeals Council that includes:
  - Your name and Medicare Number. If you've appointed a representative, include their name.
  - The specific item(s) and/or service(s) and the specific dates of service you're appealing. Check your **MSN** or your OMHA decision for this information.
  - A statement identifying the parts of OMHA's decision you disagree with and an explanation of why you disagree. Send a copy of your request to your provider.
  - The date of OMHA's decision.
  - If you're asking that your case be moved from OMHA to the Medicare Appeals Council because OMHA hasn't issued a timely decision, send your request to OMHA at the address listed on the QIC's reconsideration notice or, the OMHA office where the request for hearing or review is pending (if you know that your case was assigned to an OMHA adjudicator).

Keep a copy of everything you send to Medicare as part of your appeal. For more information about the Appeals Council review process, visit **HHS.gov/about/agencies/omha/the-appeals-process/level-4/index.html**. If you need help filing a request for Medicare Appeals Council review, call 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.

You can request judicial review in Federal district court (Level 5) if:

- The amount of your case meets a minimum dollar amount. For 2026, the minimum dollar amount is $1,960. You may be able to combine claims to meet this dollar amount.

  AND

- The Medicare Appeals Council doesn't issue a timely decision.

  OR

- You disagree with the Medicare Appeals Council's decision in Level 4.

You have 60 days after you get the Appeals Council's decision or notice to ask for judicial review by a federal district court.

Level 5: Judicial Review in Federal District Court

How do I ask for a judicial review?

Follow the directions in the Medicare Appeals Council's letter you got in Level 4 to file an appeal in Federal District Court.

For more information on the appeals process

- Visit **Medicare.gov/appeals**.
- Call 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.
- Visit **Medicare.gov/basics/forms-publications-mailings/forms/appeals** for appeals forms.
- Call your **State Health Insurance Assistance Program (SHIP)** for free, personalized health insurance counseling, including help with appeals. To get the phone number for the SHIP in your state, visit **shiphelp.org**, or call 1-800-MEDICARE.

What's an "Advance Beneficiary Notice of Non-coverage" (ABN)?

If you have **Original Medicare** and your doctor, other **health care provider**, or **supplier** thinks that Medicare probably (or certainly) won't pay for items or services, he or she may give you a written notice called an ABN (Form CMS-R-131).

The ABN lists the items or services that Medicare isn't expected to pay for, an estimate of the costs for the items and services, and the reasons why Medicare may not pay. The ABN gives you information to make an informed choice about whether to get items or services, **understanding that you may have to pay for them**.

You'll be asked to choose an option box and sign the notice to say that you read and understood it. You must choose one of these options:

- **Option 1—**You want the items or services that Medicare may not pay for. Your provider or supplier may ask you to pay for them now, but you also want your provider or supplier to submit a **claim** to Medicare for the items or services.

  If Medicare denies payment of the claim, you're responsible for paying, but you can **appeal** the denied claim to Medicare. If Medicare does pay, the provider or supplier will refund payments you made (minus the **copayments** and **deductibles** you paid).
- **Option 2—**You want the items or services that Medicare may not pay for, but you don't want your provider or **supplier** to submit a claim to Medicare. You may be asked to pay for the items or services now, but because you asked your provider or supplier not to submit a claim to Medicare, you can't file an appeal.
- **Option 3—**You don't want the items or services that Medicare may not pay for, and you aren't responsible for any payments. A claim isn't submitted to Medicare, and you can't file an appeal.

An ABN isn't an official denial of coverage by Medicare. If payment is denied when a claim is submitted, you have the right to file an appeal.

Other types of Advance Notices of Non-coverage

- **"Skilled Nursing Facility Advance Beneficiary Notice of Non-coverage"**

  A skilled nursing facility will issue you a "Skilled Nursing Facility Advance Beneficiary Notice of Noncoverage" (Form CMS-10055) if there's reason to believe that Medicare may not cover or continue to cover your care or stay because it isn't reasonable and necessary, or is considered non-medical long-term care.

  If you choose to get services that Medicare Part A (Hospital insurance) may not cover, you don't have to pay for these services until a claim is submitted and Medicare officially denies payment. However, while the claim is processed, you have to continue paying costs that you would normally have to pay, like the daily coinsurance and costs for services and supplies Medicare generally doesn't cover.

  The skilled nursing facility may use the "Skilled Nursing Facility Advance Beneficiary Notice of Non-coverage" and collect money from you now for Medicare Part B (Medical insurance) items or services. If Medicare pays, the skilled nursing facility will refund any payments you made, except copayments or deductibles.

- **"Hospital Issued Notice of Non-coverage" (HINN)**

  Hospitals use a "Hospital Issued Notice of Non-coverage" when Medicare may not cover all or part of your inpatient hospital care, or, in some cases, related Part B services. This notice will tell you why the hospital thinks Medicare won't pay, and what you may have to pay if you keep getting these services. There are 4 types of "Hospital Issued Notices of Non-coverage:"

  - Preadmission/Admission
  - Hospital Requested Review
  - Noncovered Services During Covered Stay
  - Noncovered Continued Stay

Services & supplies Medicare generally doesn't cover

Doctors, other health care providers, and suppliers don't have to (but still may) give you an ABN for services that Medicare generally doesn't cover, like:

- Dental services
- Hearing aids
- Routine eye exams
- Routine foot care

What notices do home health agencies give?

Home health agencies are required to give people with Original Medicare written notices including:

"Home Health Change of Care Notice"

Your home health agency should give you this written notice when your home health plan of care is changing because of one of these:

- The home health agency makes a business decision to reduce or stop giving you some or all of your home health services or supplies
- Your doctor changed your orders, which may reduce or stop giving you certain home health services or supplies

The "Home Health Change of Care Notice" lists the services or supplies that will be changed, and it gives you instructions on what you can do if you don't agree with the change.

The home health agency isn't required to give you a "Home Health Change of Care Notice" when it issues the "Notice of Medicare Non-Coverage."

"Advance Beneficiary Notice of Non-coverage"

When a home health agency believes that Medicare may not pay for certain home health items and services or all of your home health care, the agency should give you an "Advance Beneficiary Notice of Non-coverage" (ABN). Go to page 13 for more information on the ABN.

Home health agencies are required to give you an ABN if your care is reduced or stopped, or before you get any items or services that Medicare may not pay for because:

- They're not considered medically reasonable and necessary
- The care is non-medical long-term care
- You aren't confined to your home
- You don't need intermittent skilled nursing care

"Notice of Medicare Non-Coverage"

Your home health agency will give you this notice when all of your Medicare-covered services are ending. This notice will tell you when the services will end and how to appeal if you think the services are ending too soon. This notice also tells you how to contact your Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO) to ask for a fast appeal. If you don't get this notice, ask for it. For more information your BFCC-QIO visit CMS.gov/medicare/quality/quality-improvement-organizations/family-centered-care.

If you decide to ask for a fast appeal, you should call the BFCC-QIO within the timeframe listed on the notice. After you ask for a fast appeal, you'll get a second notice with more information about why your care is ending. The BFCC-QIO may ask you about your care. To help your case, ask your doctor for information to submit to the BFCC-QIO.

"Detailed Explanation of Non-Coverage"

Your home health agency will give you this notice when it's informed by the BFCC-QIO that you've asked for a BFCC-QIO review of your case. The notice will explain why your home health agency believes that Medicare will no longer pay for your home health care.



Section 4:

File an appeal with a Medicare Health Plan

Medicare Advantage Plans and Medicare Cost Plans are health plan options approved by Medicare and run by private companies. When you join a Medicare Advantage Plan or other **Medicare health plan**, you're still in Medicare.

If you have a Medicare Advantage Plan or other Medicare health plan, you have the right to ask your plan to provide or pay for items or services you think it should cover, provide, or continue. You also have the right to ask for an **appeal** to resolve differences with your plan. Your Medicare Advantage Plan or other Medicare health plan will send you information that explains your rights. Call your plan if you have questions.

Note: Go to pages 40–43 for definitions of **blue** words

Medicare Cost Plans are types of Health Maintenance Organizations (HMOs) available in certain areas of the country. If you have a Medicare Cost Plan and want to appeal services you got outside of the plan's network, you'll follow the **Original Medicare** appeals process. Go to page 5.

If you're in a Program of All-inclusive Care for the Elderly (PACE), your **appeal** rights may be different. The PACE organization will give you written information about your appeal rights.

If you decide to file an appeal, ask your doctor, other health care provider, or **supplier** for any information that may help your case. **Keep a copy of everything you send to your plan as part of your appeal**.

How do I file an appeal in a Medicare Advantage Plan?

Request an organization determination

You have the right to ask your plan to provide or pay for items, services, or Part B drugs you think it should cover, provide, or continue. The decision by the plan is called an "organization determination." You, your representative, or your health care provider can ask for an organization determination from your plan in advance to make sure that the services are covered. Your plan must process your request for advance coverage within 72 hours for Part B drugs, 7 days for items and services that your plan requires prior authorization, or 14 days for all other items and services. These organization determinations must be processed as quickly as your health requires. In some cases, the time to complete appeals may be extended by up to 14 days. (For example, if your plan needs more information from a non-contract provider to make a decision about your case, and the extension is in your best interest.) Your plan will notify you in writing if it decides to take an extension, why, and your rights if you disagree with their decision.

If you think your health could be seriously harmed by waiting the standard timeframe for a decision, depending on the type of request (or 72 hours for a request for Part B drugs), ask your plan for a fast or "expedited" decision. The plan must tell you of its decision within 24 hours for Part B drugs or 72 hours for items and services when it determines, or your doctor tells your plan, that waiting for a standard decision may seriously jeopardize your life, health, or ability to regain maximum function.

After you get an item, service, or Part B drug that you think your plan should cover, you or your health care provider may ask your plan pay for it. Your plan must process these payment requests within 60 days.

If the plan won't cover the items, services, or Part B drugs you asked for, you'll get a notice explaining why your plan fully or partially denied your request, and instructions on how to appeal your plan's decision by asking for a reconsideration. If you appeal the plan's decision, you may want to ask for a copy of your file containing medical and other information about your case. Your plan may charge you for this copy.

If you disagree with your plan's organization determination, you can file an appeal. The appeals process has 5 Levels:

Level 1: Reconsideration from your plan

Level 2: Review by an Independent Review Entity (IRE)

Level 3: Decision by the Office of Medicare Hearings and Appeals (OMHA)

Level 4: Review by the Medicare Appeals Council (Appeals Council)

Level 5: Judicial Review in a Federal District Court

If you disagree with the decision made at any level of the appeals process, you can generally go to the next level. At each level, you'll get instructions on how to move to the next level of appeal.

Level 1: Reconsideration from your plan

Level 1 appeals in a **Medicare Advantage Plan** are called "Health Plan Reconsiderations." If you disagree with the initial decision from your plan, you or your representative can ask for a reconsideration. Follow the directions in the plan's initial denial notice and plan materials to start your appeal. If your appeal is for a service you haven't gotten yet, your doctor can ask for a reconsideration on your behalf (and must notify you about it).

You, or your representative, must file an **appeal** within 65 days from the date on the initial denial notice sent by your plan. If you miss the deadline, you must give a reason for filing late.

Include this information in your appeal:

- Your name, address, and the Medicare Number on your Medicare card
- The items or services you're appealing, the dates of service, and the reason(s) why you're appealing.
- The name of your representative and proof of representation, if you've appointed a representative.
- Any other information (like a doctor's note) to make your appeal stronger.

You should also include any other information that may help your case. Keep a copy of everything you send to your plan as part of your appeal.

If you think your health could be seriously harmed by waiting the standard processing time for a decision, ask your plan for a fast or "expedited" appeal. The plan must give you its decision within 72 hours if it determines, or your doctor tells your plan, that waiting for a standard decision may seriously jeopardize your life, health, or ability to regain maximum function.

How long your plan has to respond to your initial appeal depends on the type of appeal you're filing:

Standard process:

- Pre-service appeal: 30 days*
- Payment appeal: 60 days
- Part B drugs appeal: 7 days*

Fast appeal: 72 hours

*These reconsiderations must be processed as quickly as your health requires.

The time to complete appeals may be extended by up to 14 days in some cases. (For example, if your plan needs more information from a non-contract provider to make a decision about your case, and the extension is in your best interest.) Your plan will notify you in writing if it decides to take an extension, why, and your rights if you disagree with their decision.

If the plan decides against you (fully or partially), your appeal is **automatically** sent to Level 2.

Level 2: Review by an Independent Review Entity (IRE)

If your plan upholds their denial in your Level 1 appeal, they'll automatically forward their reconsideration decision to an Independent Review Entity (IRE) to start a Level 2 appeal.

How long the IRE has to respond depends on the type of appeal.

Standard process:

- Pre-service appeal: 30 days
- Payment appeal: 60 days
- Part B drugs: 7 days

Fast appeal: 72 hours

The time to complete appeals may be extended by up to 14 days in some cases. For example, if the IRE needs more information from your plan to make a decision about the case, and the extension is in your best interest. The IRE will notify you in writing if it decides to take an extension.

You'll get a fast decision if the IRE determines that your life or health may be at risk by waiting for a decision.

If you disagree with the IRE's decision in Level 2:

You have 60 days from the date you get the IRE's decision to ask for a Level 3 appeal, which is a decision by OMHA.

Level 3: Decision by the Office of Medicare Hearings and Appeals (OMHA)

If you file an appeal with OMHA, the amount of your case must meet a minimum dollar amount. For 2026, the minimum dollar amount is $200.

You can ask for a hearing before an Administrative Law Judge (ALJ) or, in certain circumstances, if you don't want to have a hearing, you can ask for an on-the-record review of your appeal by an ALJ or attorney adjudicator. To ask for a hearing before an ALJ, follow the directions on the "Medicare Reconsideration Notice" you got from the IRE in your Level 2 appeal.

A hearing before an ALJ allows you to present your appeal to a new person who will independently review the facts of your appeal and listen to your testimony before making a decision. An ALJ hearing is usually held by phone or video-teleconference, but can also be held in person if the ALJ finds that you have a good reason.

You or your representative can ask for a hearing in **one of these ways**:

- Fill out a **"Request for Medicare Hearing by an Administrative Law Judge" form**.
- Submit a written request, which must include:
  - Your name, address, phone number, and Medicare Number. If you've appointed a representative, include the name, address, and phone number of your representative.
  - The appeal number assigned by the IRE if any.
  - The dates of service for the items or services you're appealing.
  - Why you disagree with the IRE's decision.
  - Any information to make your appeal stronger. If you can't include this information with your request, include a statement explaining what you plan to submit and when you'll submit it.

File an appeal online with OMHA by visiting **HHS-ecape-portal.entellitrak.com**.

Get more information about the ALJ hearing process at **HHS.gov/about/agencies/omha/filing-an-appeal/faqs/requesting-an-alj-hearing**, or call us at 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.

How do I ask for a review of my case without a hearing?

You can ask OMHA to make a decision without holding a hearing (based only on the information that's in your appeal record). The ALJ or attorney adjudicator may also issue a decision without holding a hearing if the appeal record supports a decision that's fully in your favor.

To ask OMHA to make a decision without a hearing (based on only the information that's in your appeal record), submit the information required for an ALJ hearing (listed above) and one of these:

- The"Waiver of Right to an Administrative Law Judge (ALJ) Hearing" form (Form OMHA-104). To download the form, visit **HHS.gov/about/agencies/omha/filing-an-appeal/forms**.
- A written request stating that you don't wish to appear before an ALJ at a hearing (including a hearing held by phone or video-teleconference).

Even if you waive the ALJ hearing, a hearing may still be held by an ALJ if the other parties in your case who were sent a notice of hearing (for example, your provider) don't also waive the ALJ hearing, or if the ALJ believes a hearing is necessary to decide your case.

If you asked OMHA for a decision without a hearing, but the ALJ decides a hearing is necessary, the ALJ will let you know when the hearing will be. If no hearing is held, either an ALJ or attorney adjudicator will review the information in your appeal record and make a decision.

You can ask to move to appeals Level 4 if:

- OMHA doesn't issue a timely decision.
- You disagree with the OMHA's decision.

You have 60 days after you get the decision to ask for appeals Level 4. You'll follow the directions in the ALJ's hearing decision.

Level 4: Review by the Medicare Appeals Council

You can ask that the Medicare Appeals Council review your case regardless of the dollar amount.

You can ask for the Medicare Appeals Council (Appeals Council) review in 1 of 2 ways:

- Fill out a "Request for Review of an Administrative Law Judge (ALJ) Medicare Decision/Dismissal" form: **HHS.gov/sites/default/files/dab101.pdf**.
- Submit a written request to the Appeals Council that includes:
  - Your name and Medicare Number. If you've appointed a representative, include the name of your representative.
  - The specific item(s) and/or service(s) and specific date(s) of service you're appealing.
  - A statement describing what you disagree with in the ALJ's decision and why.
  - The date of the ALJ decision.
  - If you're asking that your case be moved from the ALJ to the Appeals Council because the ALJ hasn't issued a timely decision, send your ask to the OMHA address listed on the IRE's reconsideration notice (or, if you know that your case was assigned to an OMHA adjudicator, to the hearing office where it's pending).

For more information about the Medicare Appeals Council review process, visit HHS.gov/about/agencies/dab/different-appeals-at-dab/appeals-to-council, or call us at 1-800-MEDICARE (1-800-633-4227).

Level 5: Judicial Review in a Federal District Court

You can request judicial review in Federal district court (level 5) if:

- The Appeals Council doesn't issue a timely decision.
- You disagree with the Appeals Council's decision in Level 4.
- The amount of your case meets a minimum dollar amount. For 2026, the minimum dollar amount is $1,960. You may be able to combine claims to meet this dollar amount.

You have 60 days after you get the Appeals Council's decision to ask for judicial review by a federal district court.

How do I request a review?

Follow the directions in the Appeals Council's decision letter you got in Level 4 to file a complaint in Federal District Court.

For more information on the appeals process

- Visit Medicare.gov/providers-services/claims-appeals-complaints/appeals/medicare-health-plans.
- Call 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048. Visit Medicare.gov/basics/forms-publications-mailings/forms/appeals for appeal forms.
- Call your State Health Insurance Assistance Program (SHIP) to get help with appeals.
  To get the phone number for the SHIP in your state, visit shiphelp.org, or call 1-800-MEDICARE.

How do I file a complaint (grievance)?

If you have concerns or problems with your Medicare Advantage Plan or other Medicare health plan that don't involve providing or paying for items or services, you can file a complaint, also known as a grievance.

- If your concern involves the quality of care you got or are getting, you can file a complaint with your plan and/or your BFCC-QIO. You can visit CMS.gov/medicare/quality/quality-improvement-organizations/family-centered-care or call 1-800-MEDICARE to get the BFCC-QIO's phone number.
- For example, you may file a complaint with your Medicare health plan if:
  - You believe your plan's customer service hours of operation should be different.
  - You believe there aren't enough specialists in the plan to meet your needs (but you're not asking to visit an out-of-network specialist, which requires an organization determination).

- The company offering your plan is sending you materials that you didn't ask to get and aren't related to your plan.
- The plan didn't make a decision about a reconsideration within the required timeframe.
- The plan didn't send your case to the IRE.
- You disagree with the plan's decision not to grant your request for a fast appeal, or you disagree with the plan's decision to extend the timeframe for making its decision.
- The plan didn't provide the required notices.
- The plan's notices don't follow Medicare rules.

When you join a Medicare Advantage Plan or other Medicare health plan, the plan will send you information about how to file complaints in its membership materials. Read the information carefully, and keep it where you can find it if you need it. Call your plan if you have questions.



Section 5:

Your right to a fast appeal

You have the right to a fast appeal

You have the right to a fast appeal in some situations.

If you're a hospital inpatient and the hospital decides that your inpatient stay is ending, they'll give you a written notice called "An Important Message from Medicare about Your Rights" (sometimes called the "Important Message from Medicare" or the "IM"). You'll get this notice before your services end, and it will tell you how to ask for a fast appeal. If they don't give you this notice, ask for it. This notice lists the **Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO)'s** contact information and explains:

- Your right to get all medically necessary hospital services
- Your right to be involved in any decisions that the hospital, your doctor, or anyone else makes about your hospital services and to know who will pay for them
- Your right to get the services you need after you leave the hospital
- Your right to appeal a discharge decision and the steps for appealing the decision

Note: Go to pages 40–43 for definitions of **blue** words

- The circumstances under which you will or won't have to pay for charges for continuing to stay in the hospital
- Information on your right to get a detailed notice about why your covered services are ending

You may have the right to ask for a fast appeal if you think your services from one of these providers are ending too soon:

- Hospital
- **Skilled nursing facility (SNF)**
- Home health agency (HHA)
- Comprehensive outpatient rehabilitation facility (CORF)
- Hospices

Your provider will give you a Notice of Medicare Non-Coverage (NOMNC) telling you how to ask for a fast appeal. You should read this notice carefully. If you don't get this notice, ask your provider for it. With a fast **appeal**, an independent reviewer, called a **Beneficiary and Family Centered Care-Quality Improvement Organization (BFCC-QIO)**, will decide if your covered services should continue.

What do I need to know before I ask for a fast appeal?

In a hospital: Within 2 days of your admission and before your discharge, you should get a notice called "An Important Message from Medicare about Your Rights." This notice is sometimes called the "Important Message from Medicare" or the "IM". If you don't get this notice, ask for it.

If the hospital gives you the IM more than 2 days before your discharge day, it must give you a copy of your original, signed IM or give you a new one (that you must sign) before you're discharged.

In other settings: You should get a notice called "Notice of Medicare Non-Coverage" at least 2 days before covered services end. If you don't get this notice, ask for it.

In hospitals or other settings the notice you get will include:

- Your right to appeal and the steps for appealing.
- The circumstances under which you will or won't have to pay for services you get after the coverage end date on your notice.
- The date your covered services will end.
- Information on your right to get a detailed notice about why your covered services are ending.
- Your right to a fast appeal and information on how to contact the Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO) in your state. The BFCC-QIO is administered for Medicare by Livanta or Acentra, depending on what state you live in. Check **Commencehealthqio.cms.gov/** or **Acentraqio.com/** to make sure you're contacting the right organization for your state.

How do I ask for a fast appeal?

In a hospital: Follow the directions on the "Important Message from Medicare" no later than the day you're scheduled to be discharged from the hospital.

If you ask for your **appeal** within this time frame, you can stay in the hospital while you wait to get the **Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO)** decision. You won't have to pay for your stay (except for applicable coinsurance or deductibles). If you miss the deadline for a fast appeal, you can still ask the BFCC-QIO to review your case, but different rules and time frames apply, and you might be responsible for the cost of the hospital stay past the original day the hospital tries to discharge you.

In other settings: Follow the instructions on the "Notice of Medicare Non-Coverage" no later than noon the day before the termination date listed on the notice.

If you miss the deadline to ask for a fast appeal from the BFCC-QIO, you can ask for a fast reconsideration from your plan, but services will only be covered if there's a decision issued in your favor.

What will happen during the BFCC-QIO's review?

In a hospital: The BFCC-QIO will notify the hospital. If you have a Medicare health plan, the BFCC-QIO will notify your plan and the hospital. Then, by noon the day after the BFCC-QIO notifies the hospital (or your plan), the hospital will give you a "Detailed Notice of Discharge."

In other settings: The BFCC-QIO will notify the provider. Then, by the end of the day that the provider gets the notice from the BFCC-QIO, the provider will give you a "Detailed Explanation of Non-Coverage."

In hospitals or other settings the notice you get will include:

- Why your services are no longer reasonable and necessary or are no longer covered
- A description of the applicable Medicare coverage rule or policy, including information on how you can get a copy of the policy
- How the applicable coverage rule or policy applies to your specific situation

You can also ask for copies of any of the materials that were sent to the BFCC-QIO.

In all cases, the BFCC-QIO will:

- Ask why you believe coverage for the services should continue
- Look at your medical records and the information provided by the hospital, provider or your plan
- Make a decision:
  - In a hospital: Within one day of getting the requested information
  - In other settings: By close of business the day after it gets the information it needs to make a decision

What does the BFCC-QIO decision mean for my appeal?

In a hospital:

- If they decide your services are **ending too soon**: Medicare will continue to cover (except for applicable coinsurance or deductibles) your hospital stay as long as medically necessary. If you get any inpatient hospital services after noon of that day, you may have to pay for them.
- If they decide your services **should end**: If you met the deadline for asking for a fast appeal, you won't be responsible for paying the hospital charges (except for applicable coinsurance or deductibles) you have through noon of the day after the BFCC-QIO gives you its decision. If you get any impatient hospital services after noon that day, you may have to pay for them.

In other settings:

- If they decide your services are **ending too soon:** Depending on your care needs, Medicare may continue to cover your skilled nursing facility, home health agency, comprehensive outpatient rehabilitation facility, or hospice services (except for applicable coinsurance or deductibles).
- If they decide your services **should end:** You won't be responsible for paying for any skilled nursing facility, home health agency, comprehensive outpatient rehabilitation facility, or hospice services provided before the coverage end date on the "Notice of Medicare Non-Coverage." If you continue to get services after the coverage end date, you may have to pay for those services.

If you have any questions about fast appeals, call the BFCC-QIO at the phone number listed on the notice you get from the facility. You can also visit CMS.gov/medicare/quality/quality-improvement-organizations/family-centered-care or call 1-800-MEDICARE (1-800-633-4227) to get the BFCC-QIO's phone number. TTY users can call 1-877-486-2048.

Appeal when a hospital changes your status from "inpatient" to "outpatient getting observation services"

You have the right to ask for a fast appeal if you were admitted as a hospital inpatient, and your status was changed to "outpatient getting observation services" during your hospital visit. Your hospital status affects what you pay, and if Medicare will cover skilled nursing care after you leave the hospital.

If your status is changed from "inpatient" to "outpatient getting observation services" during your hospital stay, you should get a "Medicare Change of Status Notice (CMS-10868)" before you leave the hospital. If you don't get this notice, ask for it. This notice explains:

- Your hospital stay will be billed to **Medicare Part B** instead of **Part A**. Depending on the Part B coinsurance you owe as an outpatient, your hospital bill may be lower or higher than the Part A inpatient deductible.
- Your stay will be billed as an outpatient if you don't have Medicare Part B. This means the hospital will charge you the full cost of your outpatient hospital stay.
- Medicare won't pay if you go to a **skilled nursing facility** after you leave the hospital.
- Your right to a fast appeal and information on how to contact your state **BFCC-QIO** to ask for a fast appeal. The BFCC-QIO is administered for Medicare by Commence Health or Acentra, depending on what state you live in. Check their websites to make sure you're contacting the right organization for your state.
- If you have questions about billing, ask your hospital for more information.

What happens after I file an appeal?

When the BFCC-QIO gets your appeal, the BFCC-QIO will:

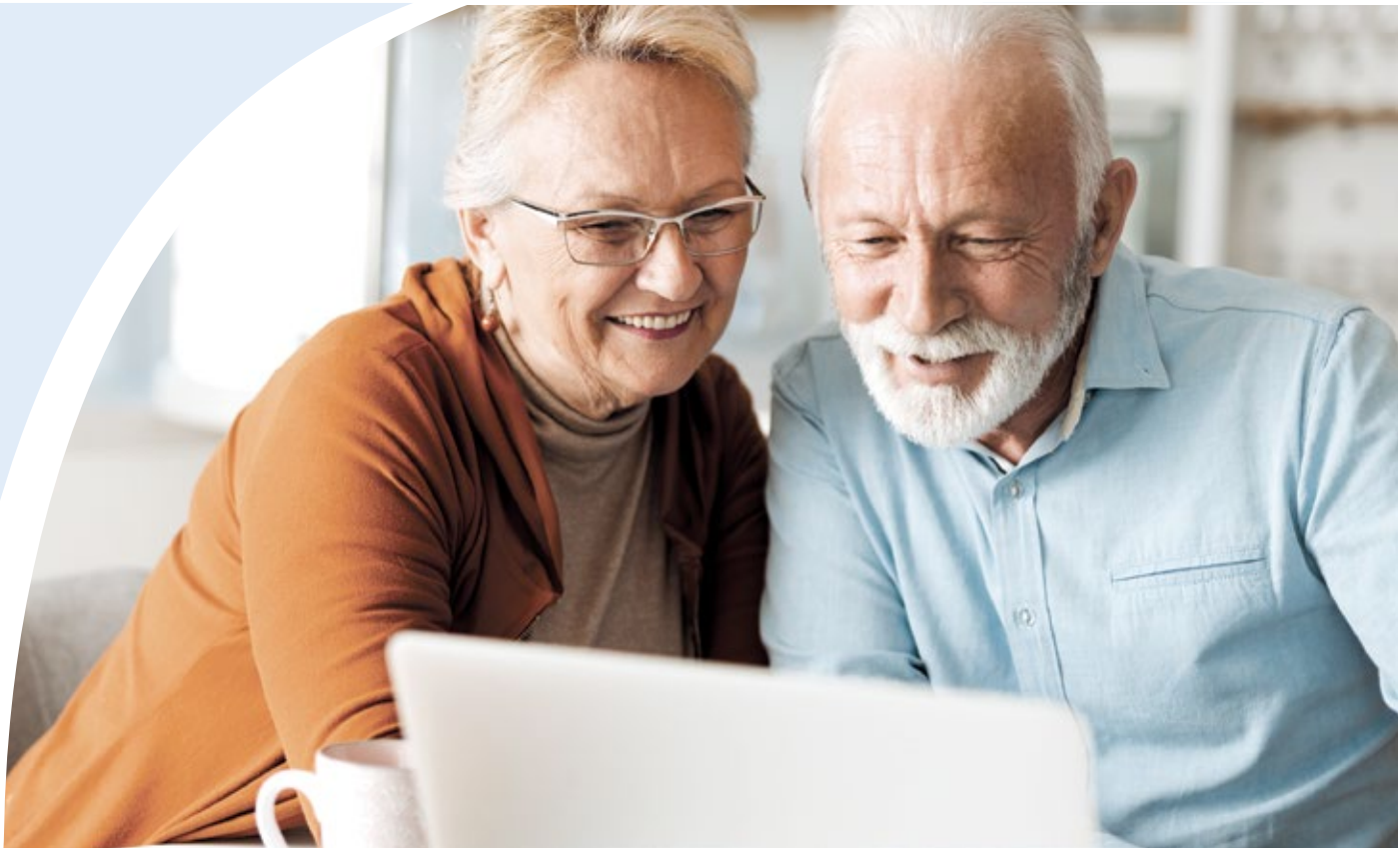
- Let the hospital know that you filed an **appeal**.
- Ask the hospital for your medical records and any relevant information.
- Give the hospital a chance to explain why the change in status from an inpatient to an outpatient getting observation services is appropriate.
- Look at your medical records and the information provided by the hospital.
- Make a decision and let you know what they decided about 2 days after you file your appeal.

You can ask for copies of any of the materials that were sent to the BFCC-QIO.

What happens when my appeal is decided?

The BFCC-QIO will notify you and the hospital once your appeal is decided.

- If they decide that your status **shouldn't** have changed: You'll be responsible for paying the Medicare Part A inpatient hospital deductible. You may qualify for a Medicare-covered skilled nursing facility stay within 30 days of your discharge from this hospital stay (if other coverage requirements are also met).
- If they decide that your status **should** have changed: You'll be responsible for paying the Medicare Part B costs (if you have Medicare Part B). If you don't have Medicare Part B, you may be responsible for the full cost of your outpatient hospital stay. You won't qualify for a Medicare-covered skilled nursing facility stay after you leave the hospital.



Section 6:

File an appeal with Medicare drug coverage

If you have Medicare drug coverage through a **Medicare drug plan**, a **Medicare Advantage Plan** with drug coverage (also known as MA-PDs), or other Medicare plan, your plan will send you information that explains your rights (called an "Evidence of Coverage"). Call your plan if you have questions about your "Evidence of Coverage."

You have the right to ask your plan to provide or pay for a drug you think it should cover, provide, or continue. You have the right to ask for an **appeal** if you disagree with your plan's decision about whether to provide or pay for a drug.

Note: Go to pages 40–43 for definitions of **blue** words

If your plan has limited your access to certain medications under its drug management program, you can appeal the plan's decision. If you **appeal**, your plan will review your case and give you a decision.

If you decide to appeal, ask your doctor or other health care provider for any information that may help your case. Keep a copy of everything you send to your plan as part of your appeal.

What if my plan won't cover a drug I think I need?

If your pharmacist tells you that your Part D plan won't cover a drug you think it should cover, or it will cover the drug at a higher cost than you think you should have to pay, they will give you a notice that explains how to ask for a coverage determination. You have these options:

- **Talk to your prescriber.**

  Ask your prescriber if you meet prior authorization or step therapy requirements. For more information on these requirements, visit **Medicare.gov,** or call 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048. You can also ask your prescriber if there are generic, over-the-counter, or less expensive brand-name drugs that could work just as well as the ones you're taking now.

- **Ask for a coverage determination (including "exceptions").**

  You, your representative, your doctor, or other prescriber can ask (orally or in writing) your plan to cover the prescription you need. You can ask for a coverage determination if your pharmacist or plan tells you one of these:

  - A drug you believe should be covered isn't covered
  - A drug is covered at a higher cost than you think you should have to pay
  - You have to meet a plan coverage rule (like prior authorization) before you can get the drug
  - It won't cover a drug on your plan's drug list (also called the formulary)

You, or your representative can ask for an "exception" if:

- You think your plan should cover a drug that's not on its formulary because the other treatment options on your plan's formulary won't work for you
- Your doctor or other prescriber believes you can't meet one of your plan's coverage rules, like prior authorization, step therapy, or quantity or dosage limits
- You think your plan should charge a lower amount for a drug you're taking on the plan's non-preferred drug tier because the other treatment options in your plan's preferred drug tier won't work for you

If you ask for an exception, your doctor or other health care provider who prescribed the medicine will need to give a supporting statement to your plan explaining why you need the drug. Check with your plan to find out if the supporting statement must be made in writing. The plan's decision-making time period begins once your plan gets the supporting statement.

You can either ask for a coverage determination before you pay for your prescriptions, or you can decide to pay for the prescription, save your receipt, and then ask for the plan to pay you back by asking for a coverage determination. Check the "Evidence of Coverage" you get from your plan for more information on getting reimbursed for out-of-pocket costs.

You can ask for a standard coverage determination, or you can file an expedited (fast) request if you haven't already paid for the drug yourself.

How do I ask for a standard coverage determination?

You, your representative, your doctor, or other prescriber can ask for a coverage determination (including an exception) by filing a request with your plan. A request should be made in writing using the 'Model Coverage Determination Request' form. Some plans may allow you to make a request orally. Once your plan gets your standard coverage determination request, they must give you a decision within 72 hours of receipt of your request. For an exception request, they must give you a decision within 72 hours of receipt of the doctor's supporting statement. They will give you a decision within 14 calendar days for a decision about payment.

How do I ask for an expedited (fast) coverage determination?

You, your representative, your doctor, or other prescriber can call or write your plan to ask that a fast decision be made within 24 hours of your request. If you're asking for an exception, your plan will give you an answer within 24 hours of getting your doctor's supporting statement. You'll get a fast decision if your plan determines, or your doctor or other prescriber tells your plan, that waiting 72 hours for a decision may seriously jeopardize your life, health, or ability to regain maximum function. You can't request (and won't get) a fast decision if you've already paid for and gotten the drug.

You can call your plan or write them a letter. You can ask them to send you a "Model Coverage Determination Request" form to ask your plan for a fast coverage determination or exception.

What if I disagree with the decision?

Your Part D plan will send you a written decision. If you disagree with this decision, you have the right to file an **appeal**.

What's the appeals process for Medicare drug coverage?

The appeals process has 5 Levels:

Level 1: Redetermination from your plan

Level 2: Reconsideration by an Independent Review Entity (IRE)

Level 3: Decision by the Office of Medicare Hearings and Appeals (OMHA)

Level 4: Review by the Medicare Appeals Council (Appeals Council)

Level 5: Judicial Review by a Federal District Court

If you disagree with the decision made at any level of the process, you can generally go to the next level. At each level, you'll get instructions on how to move to the next level of appeal.

Level 1: Redetermination from your plan

If you disagree with your plan's initial denial (coverage determination), you can ask for a redetermination.

You must ask for the redetermination within 65 days from the date of the coverage determination notice. If you miss the deadline, you must give a reason for filing later.

How do I ask for a redetermination?

Follow the directions in the plan's initial denial notice and plan materials. You, your representative, your doctor, or other prescriber can ask for a standard or fast redetermination. Standard requests must be made in writing, unless your plan allows you to file a standard request orally, like by phone. You'll get a fast decision if your plan determines, or your doctor or other prescriber tells your plan, that waiting for a standard decision may seriously jeopardize your life, health, or ability to regain maximum function. You can't ask for a fast redetermination if it's an appeal about payment for a drug you already got.

Your plan must accept any written request for a redetermination from you, your representative, your doctor, or other prescriber. A written appeal should include:

- Your name, address, and Medicare Number or member number.
- The name of the drug you want your plan to cover.
- Reason(s) why you're appealing.
- If you've appointed a representative, include their name and proof of representation. For more information on appointing a representative, go to page 3.

Send your appeal with any other information that may help your case, including medical records. Your plan's address and phone number are in your plan materials and will also be in any written plan decision you get.

Your plan will respond in a "Redetermination Notice" within these timeframes:

- Expedited (fast) redetermination decision: as quickly as your health condition requires, but no later than 72 hours
- Standard redetermination benefit decision: 7 days
- Standard redetermination payment decision: 14 days

If you disagree with the plan's Level 1 redetermination decision, you can ask for a reconsideration by an Independent Review Entity (IRE), which is Level 2. You must ask for the reconsideration within 65 days from the date of your redetermination notice. If you miss the deadline, you must provide a reason for filing later.

If you appeal a limitation that applies to your access to certain medications under your plan's drug management program and your plan denies your appeal, your appeal is automatically sent to the IRE. If this happens, you'll get a written notice about it.

Level 2: Reconsideration by an Independent Review Entity (IRE)

If you disagree with the plan's redetermination, you, your representative, or your doctor or other prescriber can ask for a standard or fast reconsideration by an IRE. You can't ask for a fast reconsideration if it's an **appeal** about payment for a drug you already got.

How do I ask for a reconsideration?

To ask for a reconsideration by an IRE, follow the directions in the plan's "Redetermination Notice." If your plan issues an unfavorable redetermination, it should also send you a "Request for Reconsideration" form that you can use to ask for a reconsideration. If you don't get this form, call your plan and ask for a copy.

Send your request to the IRE at the address or fax number listed in the "Redetermination Notice" from the plan that's mailed to you. You'll get a fast reconsideration if the IRE determines, or your prescriber tells the IRE, that waiting for a standard reconsideration may seriously jeopardize your life, health, or ability to regain maximum function.

Once the IRE gets the request for review, it will send you its decision in a "Reconsideration Notice" within these timeframes:

- Expedited (fast) reconsideration decision: as quickly as your health condition requires, but no later than 72 hours
- Standard reconsideration benefit decision: 7 days
- Standard reconsideration payment decision: 14 days

Note: The IRE may also call itself the "Part D QIC."

If you disagree with the IRE's decision in Level 2, you have 60 days after you get the IRE's decision to request a decision by OMHA, which is Level 3.

Level 3: Decision by the Office of Medicare Hearings and Appeals (OMHA)

If you're not satisfied with the IRE's reconsideration decision, you may ask OMHA for a hearing before an Administrative Law Judge (ALJ) or, in certain circumstances, a review of the **appeal** record by an ALJ or an attorney adjudicator.

A hearing before an ALJ allows you to present your appeal to a new person who will independently review the facts of your appeal and listen to your testimony before making a new and impartial decision. An ALJ hearing is usually held by phone or video-teleconference, but it can be held in person if the ALJ finds that you have a good reason. You can ask OMHA to make a decision without holding a hearing, based only on the information that's in your appeal record. If you do this, either an ALJ or an attorney adjudicator will review the information in your appeal record and issue a decision. The ALJ or attorney adjudicator may also issue a decision without holding a hearing if, for example, information in your appeal record supports a decision that's fully in your favor.

To get a hearing or review by OMHA, the amount of your case must meet a minimum dollar amount. The minimum dollar amount for 2026 is $200. The "Reconsideration Notice" may include a statement that tells you if your case is estimated to meet the minimum dollar amount. However, it's up to the ALJ to make the final decision. You may be able to combine **claims** to meet the minimum dollar amount.

How do I ask for a hearing?

You must send your request to the OMHA Central Operations. The address is listed in the IRE's "Reconsideration Notice." You can ask for a hearing in one of these ways:

- Fill out a "Request for Administrative Law Judge (ALJ) Hearing or Review of Dismissal" form (OMHA-100). You can get a copy by visiting **HHS.gov/about/agencies/omha/filing-an-appeal/forms/index.html**, or by calling 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.
- Submit a written request, which must include:
  - Your name, address, phone number, Medicare Number, and the name of your plan. If you've appointed a representative, include their name, address, and phone number.
  - The appeal case number included on the "Reconsideration Notice."
  - The prescription drug in dispute. Check your "Redetermination Notice" or "Reconsideration Notice" for this information.
  - The reason why you disagree with the reconsideration decision.
  - Any other information that may help your case. If you can't include this information with your request, include a statement explaining what you plan to submit and when you'll submit it.
  - If you're asking for an expedited (fast) decision, include a statement that explains this.

- If you're asking for an expedited (fast) decision, you can make an oral request. Follow the instructions in the IRE's "Reconsideration Notice" to do this. The ALJ or attorney adjudicator may give you a fast decision if your doctor or other prescriber says, or the ALJ or attorney adjudicator determines, that using the standard 90-day timeframe for making a decision may seriously jeopardize your life, health, or ability to regain maximum function. You can't ask for (and won't get) a fast decision if you already got the drug.

  If you ask for an expedited (fast) decision, you'll get a decision as quickly as your health condition requires, but generally no later than 10 days beginning on the day OMHA gets your request for a hearing.

How do I ask for review of my case without a hearing?

To ask that OMHA make a decision without a hearing based only on the information that's in your **appeal** record, submit the information required for an ALJ hearing to the IRE **and** include either:

- The "Waiver of Right to an Administrative Law Judge (ALJ) Hearing" (Form OMHA-104). You can get a copy by visiting **HHS.gov/about/agencies/omha/filing-an-appeal/forms/index.html**.
- A written request, or an oral request in the case of an expedited appeal, stating that you don't wish to appear before an ALJ at a hearing (including a hearing held by phone or video-teleconference), and explaining why you decided to waive the hearing.

Even if you waive the ALJ hearing, a hearing may still be held if the ALJ believes a hearing is necessary to decide your case.

If you have asked OMHA for a decision without a hearing, but the ALJ decides a hearing is necessary, the ALJ will let you know when the hearing will be. If no hearing is held, either an ALJ or attorney adjudicator will review the information in your appeal record and issue a decision.

To learn more about the OMHA review process, visit **HHS.gov/about/agencies/omha/the-appeals-process/level-3/index.html**. If you need help filing an appeal with OMHA, call 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.

If you disagree with OMHA's decision in Level 3, you have 60 days after you get OMHA's decision to ask for a review by the Medicare Appeals Council (Appeals Council), which is Level 4.

Level 4: Review by the Medicare Appeals Council (Appeals Council)

You can ask that the Appeals Council review your case, regardless of the dollar amount.

How do I ask for a review?

To ask the Appeals Council to review OMHA's decision in your case, follow the directions in the decision letter you got from OMHA in Level 3. Your request must be sent to the Appeals Council at the address listed in OMHA's decision. You or your representative can file a request for Appeals Council review in one of these ways:

- Fill out a "Request for Review of an Administrative Law Judge (ALJ) Medicare Decision/Dismissal" form (DAB-101). To get a copy, visit **HHS.gov/sites/default/files/dab101.pdf**, or call 1-800-MEDICARE (1-800-633-4227) and ask for a copy. TTY users can call 1-877-486-2048.
- Submit a written request to the Appeals Council that includes:
  - Your name, address, phone number, Medicare Number, and the name of your plan. Include your representative's name and address, if you've appointed one.
  - The prescription drug in dispute. Check your IRE "Reconsideration Notice" or your decision letter from OMHA for this information.
  - A statement identifying the parts of the ALJ's decision you disagree with and an explanation of why you disagree.
  - The ALJ appeal case number.
  - If you're asking for an expedited (fast) decision, include a statement that explains this.
- If you're asking for a fast review, you can make an oral request. Follow the instructions in OMHA's decision notice to do this. The Appeals Council may give you a fast decision if your doctor or other prescriber says, or the Appeals Council determines, that using the standard 90-day timeframe for making a decision may seriously jeopardize your life, health, or ability to regain maximum function. You can't ask for (and won't get) a fast decision if you already got the drug.

If you ask for a fast Appeals Council decision, you'll get a decision as quickly as your health condition requires, but generally no later than 10 days beginning on the day the Appeals Council gets your request, unless that time period is extended.

To learn more about the Appeals Council review process, visit **HHS.gov/about/agencies/omha/the-appeals-process/level-4**. If you need help filing a request for Appeals Council review, call 1-800-MEDICARE.

If you disagree with the Appeals Council's decision in Level 4, you have 60 days after you get the Appeals Council's decision to ask for Judicial Review by a Federal District Court, which is Level 5.

Level 5: Judicial Review by a Federal District Court

If you disagree with the decision issued by the Appeals Council, you can ask for Judicial Review in Federal District Court. To get a review, the amount of your case must meet a minimum dollar amount. The minimum dollar amount for 2026 is $1,960.

How do I ask for a review?

Follow the directions in the Appeals Council's decision letter you got in Level 4 to file an appeal in Federal District Court. You should check with the clerk's office of the Federal District Court for instructions about how to file the appeal.

For more information on the appeals process:

- Visit Medicare.gov/providers-services/claims-appeals-complaints/appeals/drug-plans.
- Call 1-800-MEDICARE.
- Call your State Health Insurance Assistance Program (SHIP) to get free, personalized health insurance counseling, including help with appeals. To get the phone number for the SHIP in your state, visit shiphelp.org, or call 1-800-MEDICARE.

How do I file a grievance (complaint)?

If you have concerns or problems with your plan that don't involve requests to provide or pay for drugs, you have the right to file a complaint (also called a "grievance"). For example:

- You believe your plan's customer service hours of operation should be different
- You have to wait too long for your prescription
- The company offering your plan is sending you materials that you didn't ask to get and aren't related to the drug plan
- Your plan didn't make a timely decision about a coverage determination in Level 1 and didn't send your case to the IRE
- You disagree with the plan's decision not to grant your request for an expedited (fast) coverage determination or Level 1 appeal (called a "redetermination")
- Your plan didn't give the required notices
- The plan's notices don't follow Medicare rules

If your complaint involves the quality of care you got or are getting, you can file a **grievance** with your plan and/or your **Beneficiary and Family Centered Quality Improvement Organization (BFCC-QIO)**. You can visit **CMS.gov/medicare/quality/quality-improvement-organizations/family-centered-care** or call 1-800-MEDICARE to get the BFCC-QIO's phone number.

If you want to file a complaint with your drug plan:

- You must file your complaint within 60 days from the date of the event that led to the complaint.
- You can file your complaint with the plan over the phone or in writing.
- You must be told of the plan's decision generally no later than 30 days after the plan gets the complaint. The timeframe for completing your complaint may be extended by up to 14 days if you ask for the extension, or if your plan justifies the need for the extension and the extension is in your best interest. Your plan will tell you in writing if it decided to take an extension and tell you why.
- If the complaint relates to a plan's refusal to make an expedited (fast) coverage determination or redetermination and you haven't bought or got the drug, the plan must tell you its decision within 24 hours after it gets the complaint.

If the plan doesn't address your complaint, call 1-800-MEDICARE.

More information on filing a complaint

- Visit **Medicare.gov/providers-services/claims-appeals-complaints**.
- Call your **State Health Insurance Assistance Program (SHIP)** to get health insurance counseling and help filing a complaint. To get the phone number of the SHIP in your state, visit **shiphelp.org**, or call 1-800-MEDICARE.



Section 7:

Definitions

Appeal—An appeal is the action you can take if you disagree with a coverage or payment decision made by Medicare, your Medicare health plan, or your Medicare drug plan. You can file an appeal if Medicare or your plan refuses to:

- Cover a health care service, supply, item, or drug you think should be covered.
- Pay for a health care service, supply, item, or drug you already got.
- Change the amount you must pay for a health care service, supply, item, or drug.
- Provide or pay for all or part of a health care service, supply, item, or drug you think you still need.
- Cover drugs like opioids and benzodiazepines. This can happen when the plan has labeled you “at-risk” under a Part D drug management program.

Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO)—A type of QIO (an organization of doctors and other health care experts under contract with Medicare) that uses doctors and other health care experts to review complaints and quality of care for people with Medicare. The BFCC-QIO makes sure there is consistency in the case review process while taking into consideration local factors and local needs, including general quality of care and medical necessity.

Claim—A request for payment that you or your health care provider submit to Medicare or other health insurance when you get items and services that you think are covered.

Coinsurance—An amount you may be required to pay as your share of the cost for services after you pay any deductibles. Coinsurance is usually a percentage (for example, 20%).

Comprehensive outpatient rehabilitation facility—A facility that provides a variety of services on an outpatient basis, including physicians' services, physical therapy, social or psychological services, and rehabilitation.

Copayment—An amount you may be required to pay as your share of the cost for a medical service or supply, like a doctor's visit, hospital outpatient visit, or prescription drug. A copayment is usually a set amount, rather than a percentage. For example, you might pay $10 or $20 for a doctor's visit or prescription drug.

Deductible—The amount you must pay for health care or prescriptions before Original Medicare, your Medicare Advantage Plan, your Medicare drug plan, or your other insurance begins to pay.

Grievance—A complaint about the way your Medicare health plan or Medicare drug plan is giving care. For example, you may file a grievance if you have a problem calling the plan or if you're unhappy with the way a staff person at the plan has behaved towards you. However, if you have a complaint about a plan's refusal to cover a service, supply, or prescription, you file an appeal.

Health care provider—A person or organization that's licensed to give health care. Doctors, physician assistants, nurse practitioners, and hospitals are examples of health care providers.

Hospice—A special way of caring for people who are terminally ill. Hospice care involves a team-oriented approach that addresses the medical, physical, social, emotional, and spiritual needs of the patient. Hospice also provides support to the patient's family or caregiver.

Medically necessary—Health care services or supplies needed to diagnose or treat an illness, injury, condition, disease, or its symptoms and that meet accepted standards of medicine.

Medicare Advantage Plan (Part C)—A type of Medicare health plan offered by a private company that contracts with Medicare. Medicare Advantage Plans provide all of your Part A and Part B benefits, with a few exclusions, for example, certain aspects of clinical trials which are covered by Original Medicare even though you're still in the plan. Medicare Advantage Plans include:

- Health Maintenance Organizations
- Preferred Provider Organizations
- Private Fee-for-Service Plans
- Special Needs Plans
- Medicare Medical Savings Account Plans

If you're enrolled in a Medicare Advantage Plan:

- Most Medicare services are covered through the plan
- Most Medicare services aren't paid for by Original Medicare
- Most Medicare Advantage Plans offer prescription drug coverage

Medicare Drug Plan (Part D)—Part D adds prescription drug coverage to Original Medicare, some Medicare Cost Plans, some Medicare Private-Fee-for-Service Plans, and Medicare Medical Savings Account Plans. These plans are offered by insurance companies and other private companies approved by Medicare. Medicare Advantage Plans may also offer prescription drug coverage that follow the same rules as Medicare drug plans.

Medicare health plan—Generally, a plan offered by a private company that contracts with Medicare to provide Part A and Part B benefits to people with Medicare who enroll in the plan. Medicare health plans include all Medicare Advantage Plans, Medicare Cost Plans, and Demonstration/Pilot Programs. Program of All-inclusive Care for the Elderly (PACE) organizations are special types of Medicare health plans. PACE plans can be offered by public or private companies and provide Part D and other benefits in addition to Part A and Part B benefits.

Medicare Part A (Hospital Insurance)—Part A covers inpatient hospital stays, care in a skilled nursing facility, hospice care, and some home health care.

Medicare Part B (Medical Insurance)—Part B covers certain doctors' services, outpatient care, medical supplies, and preventive services.

Medicare Summary Notice (MSN)—A notice you get after the doctor, other health care provider, or supplier files a claim for Part A or Part B services in Original Medicare. It explains what the doctor, other health care provider, or supplier billed for, the Medicare-approved amount, how much Medicare paid, and what you must pay.

Non-medical long-term care—Non-skilled personal care, like help with activities of daily living like bathing, dressing, eating, getting in or out of a bed or chair, moving around, and using the bathroom. It may also include the kind of health-related care that most people do themselves, like using eye drops. In most cases, Medicare doesn't pay for non-medical long-term care.

Original Medicare—Original Medicare is a fee-for-service health plan that has two parts: Part A (Hospital Insurance) and Part B (Medical Insurance). After you pay a deductible, Medicare pays its share of the Medicare-approved amount, and you pay your share (coinsurance and deductibles).

Skilled nursing facility (SNF)—A nursing facility with the staff and equipment to give skilled nursing care and, in most cases, skilled rehabilitative services and other related health services.

State Health Insurance Assistance Program (SHIP)—A state program that gets money from the federal government to give free local health insurance counseling to people with Medicare.

Supplier—Generally, any company, person, or agency that gives you a medical item or service, except when you're an inpatient in a hospital or skilled nursing facility.

CMS Accessible Communications

Medicare provides free auxiliary aids and services, including information in accessible formats like braille, large print, data or audio files, relay services and TTY communications. If you request information in an accessible format, you won't be disadvantaged by any additional time necessary to provide it. This means you'll get extra time to take any action if there's a delay in fulfilling your request.

To request Medicare or Marketplace information in an accessible format, you can:

1. **Call us:**
   For Medicare: 1-800-MEDICARE (1-800-633-4227) TTY: 1-877-486-2048
   For Marketplace: 1-800-318-2596 TTY: 1-855-889-4325
2. **Email us:** altformatrequest@cms.hhs.gov
3. **Send us a letter:**
   Centers for Medicare & Medicaid Services
   Offices of Hearings and Inquiries (OHI)
   7500 Security Boulevard, Mail Stop DO-01-20
   Baltimore, MD 21244-1850
   Attn: Customer Accessibility Resource Staff (CARS)

Your request should include your name, phone number, type of information you need (if known), and the mailing address where we should send the materials. We may contact you for additional information.

Note: If you're enrolled in a Medicare Advantage Plan or Medicare drug plan, contact your plan to request its information in an accessible format. For Medicaid, contact your State Medical Assistance (Medicaid) office.

Nondiscrimination Notice

The Centers for Medicare & Medicaid Services (CMS) doesn't exclude, deny benefits to, or otherwise discriminate against any person on the basis of race, color, national origin, disability, sex, or age in admission to, participation in, or receipt of the services and benefits under any of its programs and activities, whether carried out by CMS directly or through a contractor or any other entity with which CMS arranges to carry out its programs and activities.

You can contact CMS in any of the ways included in this notice if you have any concerns about getting information in a format that you can use.

You may also file a complaint if you think you've been subjected to discrimination in a CMS program or activity, including experiencing issues with getting information in an accessible format from any Medicare Advantage Plan, Medicare drug plan, state or local Medicaid office, or Marketplace Qualified Health Plans. There are 3 ways to file a complaint with the U.S. Department of Health & Human Services, Office for Civil Rights:

1. **Online:**
   HHS.gov/civil-rights/filing-a-complaint/complaint-process/index.html
2. **By phone:**
   Call 1-800-368-1019.
   TTY users can call 1-800-537-7697.
3. **In writing:** Send information about your complaint to:
   Office for Civil Rights
   U.S. Department of Health & Human Services
   200 Independence Avenue, SW
   Room 509F, HHH Building
   Washington, D.C. 20201

U.S. Department of Health and Human Services
Centers for Medicare & Medicaid Services
7500 Security Blvd.
Baltimore, MD 21244-1850

Official Business
Penalty for Private Use, $300

Need a copy of this booklet in Spanish?

To get a free copy of this booklet in Spanish, visit Medicare.gov or call 1-800-MEDICARE (1-800-633-4227). TTY users can call 1-877-486-2048.

Esta publicación está disponible en español. Para obtener una copia gratis, visite es.Medicare.gov o llame al 1-800-MEDICARE.

Medicare

The information in this booklet describes the Medicare Program at the time this booklet was printed. Changes may occur after printing. Visit **Medicare.gov**, or call 1-800-MEDICARE (1-800-633-4227) to get the most current information. TTY users can call 1-877-486-2048.

"Medicare Appeals" isn't a legal document. Official Medicare Program legal guidance is contained in the relevant statutes, regulations, and rulings.

This product was produced at U.S. taxpayer expense.

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